

Oncology Imaging: PET/CT Scan

Policy: POL-ONC-220 | Effective: 2026-03-01 | Oncology

COVERED PROCEDURES

- 78816 - PET imaging for CT attenuation, whole body
- 78815 - PET imaging for CT attenuation, limited area
- 78814 - PET imaging with CT, limited area

DIAGNOSIS CODES

- C34.90 - Malignant neoplasm of unspecified bronchus/lung
- C34.91 - Malignant neoplasm of right bronchus/lung
- C34.92 - Malignant neoplasm of left bronchus/lung
- C50.911 - Malignant neoplasm of unspecified site, right breast
- C50.912 - Malignant neoplasm of unspecified site, left breast
- C18.9 - Malignant neoplasm of colon, unspecified
- C20 - Malignant neoplasm of rectum
- C83.30 - Diffuse large B-cell lymphoma, unspecified
- C85.90 - Non-Hodgkin lymphoma, unspecified

MEDICAL NECESSITY CRITERIA

All of the following must be documented for approval:

CRIT-1

Confirmed or strongly suspected malignancy with pathology report or imaging findings
Threshold: pathology or imaging evidence

CRIT-2

Clinical indication: initial staging, restaging after treatment, or evaluation of suspected recurrence
Threshold: staging|restaging|recurrence

CRIT-3

Conventional imaging (CT, MRI) performed first and is insufficient for clinical decision-making
Threshold: CT or MRI completed

CRIT-4

Requesting physician is oncologist, pulmonologist, or surgeon managing the malignancy
Threshold: oncologist|pulmonologist|surgeon

CRIT-5

PET/CT will change the treatment plan (e.g., determine surgical candidacy, radiation field planning)
Threshold: documented impact on management

URGENT BYPASS

Suspected brain metastases with acute neurological decline - authorize emergent imaging.

DENIAL REASONS

- No confirmed malignancy (screening not covered)
- Conventional imaging not attempted first
- Requesting provider is not managing oncologist
- Surveillance PET within 3 months of prior PET without clinical change
- Indication is routine surveillance with no symptoms of recurrence